

Worked example — country action plan

Reference · Results Communication · ~10 min to read

WHAT THIS HANDOUT IS

A fully-filled example of a country action plan, built around one finding — **low rates of haemoglobin measurement during antenatal care**.

HOW TO USE IT

Keep it beside you while you fill in your own plan. Notice how each of the four dimensions gets a specific, concrete answer — never a vague one.

The finding

Only **42%** of lower-level health facilities systematically record haemoglobin (Hb) measurement during antenatal care visits. Risk: undetected maternal anaemia.

The pages that follow show how this one finding turns into a complete year-ahead plan, organized around the four dimensions: **use cases, finalizing the report, ongoing updates and dissemination, and skill building**.

1. Use cases

The question this country will keep answering with FASTR: *Are facilities measuring Hb at ANC, and where is the gap closing?*

Quarterly FASTR analyses to run:

- Hb-at-ANC measurement coverage by district (quarterly)
- DQA: completeness of the ANC register (quarterly)
- Disruption check on ANC1 volume (quarterly) — to rule out broader service disruption confounding the Hb story

M&E indicators tracked:

- **% of facilities recording Hb at ANC** — target $\geq 80\%$ by March 2027
- Number of facilities equipped with hemocues
- Number of providers trained on the Hb protocol
- % of quarterly reports presented in relevant forums

2. Finalising the report

The workshop output — a Hb-at-ANC disruption-report deck — becomes the artefact for the first round of dissemination.

What	Detail
What to add	2-page narrative summary · District priority list (the 10 lowest-performing districts) · Three key messages
By when	25 May, before the RMNCAH-N meeting
Lead	Project lead (M&E unit)
Validation	Director of Reproductive Health signs off on the deck and the key messages

3. Ongoing updates and dissemination

Responsibilities:

Step	Owner	Cadence
Upload month-end HMIS data	Data Manager (M&E unit)	By the 5th of each month
Draft the quarterly Hb-at-ANC report	M&E officer	First week of each quarter
Present at RMNCAH-N coordination meeting	Project lead	Quarterly
District-level dissemination	District health managers + supervision teams	Quarterly

Existing forums to plug into:

- Quarterly RMNCAH-N coordination meetings
- District performance reviews
- Monthly bulletins (PDF / email)
- District debrief workshops
- Parliament and donor reports

Calendar — the year ahead:

When	Milestone
May 2026	RMNCAH-N meeting (25 May) — present findings
June 2026	Debrief priority districts · launch training plan
September 2026	Mid-term review
December 2026	Annual review and report

4. Skill building

Activity	Who	When
Train 50 providers on Hb-at-ANC protocols	MoH Training Division	Before Q3
Train 2 more M&E team members on FASTR end-to-end	Country focal point	Before the Q3 analysis
Weekly 1-hour FASTR clinic for the M&E team	M&E unit lead	Ongoing from Q2

What to notice in this example

Three things make this plan actionable, not aspirational:

- 1. One specific person owns each step** — not "the ministry" or "the unit".
- 2. Every action is checkable** — "train 50 providers", "equip 30 facilities" — you can tell next quarter whether it happened or didn't.
- 3. The calendar uses real dates tied to existing meetings** — 25 May, June district debriefs, mid-term in September. No "as soon as possible".

Use this as a model when you fill in your own plan. If your worksheet doesn't have a specific person, a checkable action, and a real date for each dimension — it isn't ready yet.

What's next

Take this example into the **country action plan** activity. Fill in your own plan using your country's findings. Trade with another country team for peer review — ask them: would they be able to check, three months from now, whether each row got done?